

Steroid moon

ORAL X

NOT lower limb

TNF

	TNF	IL-17	IL-23	JAK
axSpA	✓	✓	X	✓
PsA	✓	✓	✓	✓
ReA	✓	-	-	-
IBD-SpA	(mth only) ✓	X	✓	✓

SHARED

axSpA PsA ReA IBD-SpA

PERIPHERAL ✓ AXIAL X

SCREEN

TB HBV HCV

1st

NSAID

24 wk

Candida

IL-17

ACTIVE IBD

SECUR/MERIME

GUSEL/RISA/UST

HAGRY/IBD

BLACK BOX

MTX SSZ LEFL

JAK1 JAK3

HEART (CVI)

CANCER (malignancy)

HERPES BLISTERS (coxsack)

AFTER TNF

EXERCISE

1st

STOP!

HeIP spine - over at

23 HeIP gut - not the spine

1. Steroid moon crossed out

WHAT YOU SEE

Moon labeled STEROID with X, ORAL X

WHAT IT ENCODES

Systemic/oral glucocorticoids are NOT effective for AXIAL spondyloarthritis (ankylosing spondylitis / axial SpA) and are avoided — unlike RA, axial disease does not respond, and chronic steroids worsen the osteoporosis/fracture risk these patients already carry from spinal ankylosis. Local injections (intra-articular, sacroiliac, enthesal) and short courses for peripheral flares or active IBD/uveitis are acceptable.

BOARD TRAP

Choosing long-term oral prednisone for an axial-SpA flare is wrong — it doesn't work for axial disease and accelerates bone loss; escalate to a biologic instead.

2. Screen door (TB/HBV/HCV)

WHAT YOU SEE

Door labeled SCREEN with TB, HBV, HCV

WHAT IT ENCODES

Before any biologic (especially TNF inhibitors) screen for latent TB (IGRA or PPD + chest X-ray) and hepatitis B/C — TNF blockade can reactivate latent TB and HBV. Treat latent TB before/at biologic start. Also check vaccination status (give inactivated vaccines; avoid LIVE vaccines on biologics) and rule out demyelinating disease and significant heart failure (NYHA III–IV) before TNF inhibitors.

3. NSAID dragon STOP

WHAT YOU SEE

Wizard/dragon with STOP sign near NSAID label

WHAT IT ENCODES

NSAIDs are first-line and uniquely effective for the axial pain/stiffness of SpA; both continuous and on-demand dosing are options (continuous may slow radiographic progression in high-disease-activity AS). Reassess after an adequate trial of ~2–4 weeks at maximum tolerated dose. Watch GI/renal/CV toxicity and remember NSAIDs can flare IBD.

4. Treat-to-target

WHAT YOU SEE

Target/dartboard labeled REMISSION, LOW ACTIVITY

WHAT IT ENCODES

Management follows a treat-to-target strategy aiming for clinical remission or at least low disease activity, monitored with validated indices (ASDAS — Ankylosing Spondylitis Disease Activity Score, preferred; or BASDAI). Escalate therapy if the target is not met.

5. 1st-line NSAID

WHAT YOU SEE

Large 1st banner over NSAID

WHAT IT ENCODES

First-line for axial SpA = a trial of at least TWO different NSAIDs, each at maximum dose for ~2–4 weeks (≥4 weeks total trial), plus exercise/physiotherapy, before escalating to a biologic. Conventional DMARDs (methotrexate, sulfasalazine) do NOT work for axial disease and are reserved for PERIPHERAL arthritis.

BOARD TRAP

Adding methotrexate for persistent axial/back pain after NSAID failure is wrong — DMARDs don't treat axial disease; the correct next step is a TNF or IL-17 inhibitor.

6. Cytokine response table

WHAT YOU SEE

Scroll table: axSpA / PsA / ReA / IBD-SpA vs TNFi, IL-17, IL-23, JAKi

WHAT IT ENCODES

Biologic efficacy is SUBTYPE-specific: TNF inhibitors work across axSpA, PsA, and IBD-SpA. IL-17 inhibitors work in axSpA and PsA but FAIL in / WORSEN IBD. IL-23 (and IL-12/23) inhibitors work for PsA and psoriasis/skin but FAIL in axial SpA. JAK inhibitors work in axSpA and PsA. This map is the highest-yield board concept on this scene.

BOARD TRAP

Two big traps live here: IL-23 inhibitors do NOT work for axial SpA, and IL-17 inhibitors are contraindicated in IBD because they precipitate/worsen bowel disease — match the drug class to the subtype, not just 'it's a biologic.'

7. TNF quiver

WHAT YOU SEE

Quiver of arrows labeled TNF

WHAT IT ENCODES

TNF inhibitors are the first-line biologic for axial SpA failing NSAIDs (adalimumab, etanercept, infliximab, golimumab, certolizumab). MONOCLONAL TNF inhibitors (adalimumab, infliximab, golimumab, certolizumab) ALSO treat IBD and uveitis; etanercept (a soluble RECEPTOR) does NOT work for IBD and is less effective for uveitis — so pick a monoclonal when there is concomitant IBD or recurrent uveitis. Screen TB/HBV first.

BOARD TRAP

Etanercept is a valid TNF inhibitor for joints/skin but does NOT control IBD and is poor for uveitis — choosing etanercept for an SpA patient with active Crohn's or recurrent uveitis is the trap; use a monoclonal TNF inhibitor.

8. IL-17 hedgehog

WHAT YOU SEE

Hedgehog labeled IL-17

WHAT IT ENCODES

IL-17A inhibitors (secukinumab, ixekizumab) and the IL-17 receptor blocker brodalumab are effective for axial SpA and PsA (excellent for skin psoriasis and enthesitis), and are a key option when TNF inhibitors fail or are contraindicated.

BOARD TRAP

IL-17 inhibitors can NEW-ONSET or worsen inflammatory bowel disease — avoid them in SpA patients with IBD (or high IBD risk); brodalumab additionally carries a suicidality boxed warning.

9. IL-23 snake

WHAT YOU SEE

Snake labeled IL-23

WHAT IT ENCODES

IL-23 inhibitors (guselkumab, risankizumab) and the IL-12/23 inhibitor ustekinumab are effective for PsA, psoriasis, and Crohn's disease — but they FAIL for axial SpA despite IL-23 sitting upstream of IL-17 (a key immunology paradox tested on boards). Useful when both skin/PsA and IBD coexist.

BOARD TRAP

IL-23 blockade is ineffective for axial spondyloarthritis — selecting guselkumab/ustekinumab for predominantly axial disease is wrong even though it helps the skin and gut.

10. Wizard with potions

WHAT YOU SEE

Wizard mixing biologic potions

WHAT IT ENCODES

Biologic selection is individualized by the dominant phenotype (axial vs peripheral vs skin vs gut), extra-articular manifestations (uveitis, IBD, psoriasis), and comorbidities/contraindications (CV/VTE/malignancy risk for JAK; IBD for IL-17; demyelination/HF for TNF). Match the drug to the whole patient.

11. JAKi BLACK BOX skull

WHAT YOU SEE

Skull and BLACK BOX, JAKi labels listing MI, clots, cancer, herpes

WHAT IT ENCODES

JAK inhibitors (tofacitinib, upadacitinib) are oral options for axSpA and PsA after TNF failure, but carry an FDA BOXED WARNING for major adverse cardiovascular events (MACE), venous thromboembolism (DVT/PE), malignancy, serious infections, and herpes zoster reactivation — caution especially in patients ≥65, smokers, or those with CV/clotting/cancer risk. Vaccinate against shingles before starting.

12. SHARED axSpA/PsA/ReA/IBD-SpA

WHAT YOU SEE

Thought cloud SHARED listing axSpA, PsA, ReA, IBD-SpA, PERIPHERAL vs AXIAL

WHAT IT ENCODES

The SpA family (axial SpA/AS, PsA, reactive arthritis, IBD-associated/enteropathic SpA) shares HLA-B27 association, enthesitis, dactylitis, axial+peripheral patterns, and uveitis — but treatment diverges by AXIAL vs PERIPHERAL predominance: axial responds to NSAIDs then biologics (not DMARDs), whereas peripheral arthritis also responds to conventional DMARDs (sulfasalazine, methotrexate).

BOARD TRAP

Conventional DMARDs help only PERIPHERAL SpA, never axial — applying an RA-style 'methotrexate first' approach to axial SpA is the recurring trap.

13. Exercise trophy

WHAT YOU SEE

Trophy labeled EXERCISE 1st

WHAT IT ENCODES

Physical therapy and regular exercise (back extension/posture, range-of-motion, aerobic activity) are a CORNERSTONE, first-line non-pharmacologic treatment for axial SpA — proven to preserve spinal mobility and function — and should be prescribed for every patient alongside drugs. Smoking cessation also slows progression.